

Coastal Medical Group

12264 El Camino Real, Suite 101, San Diego, CA 92130
Phone: 858.356.0361 Fax: 858.356.0366

INJECTION REPORT

Patient Name: Ronald "Steve" Handrop

Date of Service: Dec 01, 2023

Surgeon: Dr. Sherry Kinnaird

Anesthesia: Local

Pre-Procedure Diagnosis: Cervical radiculitis

Post-Procedure Diagnosis: Cervical radiculitis

Procedure performed: Cervical Epidural Steroid Injection - C7-T1

Indications for procedure: The patient has a history of radicular neck pain that has persisted greater than 6 weeks. The pain has failed to respond to more conservative modalities including physical therapy, time, and medications. There is evidence for cervical spine disease on imaging. Cervical MRI shows: C3-C4 disc protrusion with myelomalacia, he had seen spine surgeon previously and is referred here for a cervical epidural steroid injection. Will proceed below the C3-C4 disc at C7-T1.

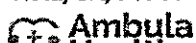
Consent: The risks, benefits, and alternative treatment options were discussed with the patient in detail. Risks include bleeding, infections, post-dural puncture headache, steroid side effects, worse pain, failure of therapy, seizure, stroke, vasovagal response, and permanent neurologic injury. The patient understands the risks and all patient questions were answered, and the patient agrees to undergo the procedure. Informed consent was verified during time-out prior to proceeding.

Procedure Description: The patient was placed in the prone position on the fluoroscopy table with appropriate padding. The patient was prepped and draped in the normal standard fashion. The C7-T1 interspace was identified and marked under AP fluoroscopy. The skin and subcutaneous tissues were anesthetized with 1% lidocaine. A 20G 3.5 inch epidural Tuohy epidural needle was directed towards the interspace under intermittent fluoroscopic guidance. A slight para-median approach from the left was taken. Once the ligamentum flavum was engaged, a loss of resistance technique with LOR saline syringe was used to identify entrance into the epidural space. A 45 degree contra-lateral oblique view was used to verify proper position during loss of resistance. Following loss of resistance, negative aspiration was confirmed and contrast was injected. Appropriate posterior epidurogram was noted. A solution consisting of 10 mg preservative free dexamethasone and 0.5 ml of preservative free normal saline was easily injected. The needle was removed with a preservative free normal saline flush.

All needles were removed intact. The skin was cleaned and a dressing was placed over the needle insertion sites. The patient tolerated the procedure well with no apparent complications. The patient was taken to the recovery room in stable condition.

Discharge instructions were given including do not get dressing wet or remove dressing for 24 hours, and limiting

Chula Vista
750 Medical Center Ct. #14 (Second Floor), Chula
Vista, CA, 91911



Patient: Ronald "Steve" Handrop
Surgeon: Dr. Kinnaird
Date of Service: 12/01/2023 Sex: M
Date of Birth: 12/07/1954 Age: 68
Medical Record: #000002501

activity for 48 hours. They are to call the office or go to the ER for any new or worsening abnormalities including severe headache, new extremity weakness/numbness/tingling, fever, or bowel/bladder problems.

Follow-up: They were instructed to have a follow-up appointment with the referral physician.

Electronically Signed by: Dr. Kinnaird

Portions of this chart may have been created with voice recognition software and may not have been edited.